

**BUSINESS ASSETS**

VYTALIX — Sales Deck for a Services Prospect (10 slides)

Vytalix company-build documentation · reference SALES_DECK

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

DATE

8 September 2026

VERSION

1.0 · Confidential draft

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Audience: a prospect for Advisory or Consulting after a discovery call (segments S1–S5 in

/docs/13_SALES_ENGINE.md §1). **Length:** 10 slides, 20 minutes, following the presentation outline in

/docs/13_SALES_ENGINE.md §6. **Version:** 1.0 · 8 September 2026 · Status: PROPOSED.

Design per /docs/03_BRAND_SYSTEM.md §13. Language per /FACTS_BASE.md §4. Bracketed fields are filled per prospect from the discovery notes (Problem / Impact / Authority / Budget / Timeline / Success criteria / Risks / Next step).

Executive view

1. The deck follows one arc — problem → approach → packages → process → next step — and is customised from discovery notes, never sent cold.
2. It sells fixed-scope, fixed-price services that exist today; MaternaLink (in development) appears only on an optional slide when the prospect's problem is maternity-related.
3. Commercials are shown as three options (Good / Better / Best) built from the catalogue in /assets/SERVICE_CATALOGUE.md and priced per /assets/PRICING_STRUCTURE.md ; all prices are ESTIMATE and exclusive of VAT.
4. Credibility is stage-honest: founder background from KNOWN facts, documented methods, no client logos, no testimonials, no invented associates.
5. The deck ends with a decision date and a kick-off date; the proposal (/assets/PROPOSAL_TEMPLATE.md) follows within two working days.

Slide 1 — Title

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- [Prospect organisation] × Vytalix
- [Engagement working title, e.g. "Getting NHS-ready in 12 weeks"]
- Prepared for [Name, role] · [Date] · Confidential — draft

Speaker note: thirty seconds on who we are: "Vytalix is a founder-led UK health-technology group — advisory, consulting, a product in development, and learning. We are pre-seed. Today is about your problem, not our story."

Visual: Deep title slide; prospect name in Bone, engagement title in Signal Mint; wordmark reverse top-left; no prospect logo without permission.

Slide 2 — Your situation, as we heard it

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- What prompted this conversation: [their words from discovery Q1].
- Where you are today: [current state; what has been tried; what happened].
- What success looks like in six months: [their success criteria, Q3].
- What happens if nothing changes by [date]: [their answer to Q6].

- Constraints we must respect from Day 1: [regulatory, clinical-safety, data-protection, procurement constraints, Q16].

Speaker note: read it back and ask "Is that right? What have I missed?" — three minutes, mostly listening.

Visual: Bone; eyebrow "WHAT WE HEARD" in Teal; four short paragraphs; the prospect's own phrases in quotation marks in Fraunces; nothing else on the slide.

Slide 3 — Why it matters now

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- The cost of the problem as you described it: [time / money / risk / missed opportunity, Q7].
- The external clock: [board meeting, funding round, tender, inspection, programme deadline, Q11].
- Sector context (cited): [one or two relevant, sourced facts — e.g. DTAC is assessed by each buying organisation, not "certified"; NHS product sales cycles of 9–18 months (ESTIMATE); MBRRACE-UK findings, current report cited; Nigeria NDPA 2023 obligations].
- Doing nothing is a decision with a price: [the consequence they named].

Speaker note: two minutes. Every fact has a source on the slide or in the notes; no fact without one.

Visual: Bone; two columns: "Your clock" and "The sector's clock"; a single stat only if sourced, set in Fraunces with the source line at 16px.

Slide 4 — How we work

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- Fixed scope, fixed price, written change control. You know the deliverable, the acceptance criterion and the date before we start.
- Founder-led. The person you met does the work; associates join only on signed work and only with your agreement.
- Evidence before claims. We cite sources; we tell you if the idea should not be built.
- Safety and data by design. Advisory and consulting engagements process no patient data.
- Every engagement leaves you a reusable asset — a template, a checklist, a model — not just a report.

Speaker note: three minutes; this is where a young firm earns trust — by being explicit about how it behaves rather than by claiming a track record.

Visual: five lines with monoline icons; whitespace; no imagery.

Slide 5 — Our proposed approach

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- Phase 1 — Understand ([n] days): [activities, e.g. 10–15 interviews, document review, current-state map]. Output: [deliverable].

- Phase 2 — Decide ([n] days): [option appraisal / gap analysis / business case draft]. Output: [deliverable].
- Phase 3 — Plan ([n] days): [90-day roadmap / readiness sequence / board paper]. Output: [deliverable].
- What we will not do: [explicit exclusions — e.g. build software, sign off a clinical safety case in place of your CSO, give legal advice, process patient data].
- Your involvement: [hours per week from the day-to-day counterpart, Q17; access to people and documents].

Speaker note: five minutes; walk the phases and the exclusions with equal weight — exclusions prevent the two most common disputes (scope and expectations).

Visual: three-column phase diagram with waypoints; Ember node on the final deliverable; exclusions in a Mist-tinted box below.

Slide 6 — Packages and options

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- Option A — Good: [package code and name, e.g. C7 Grant & Investor Readiness Sprint], [duration], [deliverables in one line], £[fee] (ESTIMATE, ex-VAT).
- Option B — Better (recommended): [e.g. C6 DTAC & Evidence Readiness Sprint + 1 advisory day per month for 3 months], [duration], [deliverables], £[fee].
- Option C — Best: [e.g. C4 UK Market-Entry Playbook + C6 + A2 retainer for 3 months], [duration], [deliverables], £[fee]; bundle pricing per `/assets/PRICING_STRUCTURE.md` \$6.
- All options: fixed fee; 50% on signing for engagements under £20k, 40/30/30 milestones above; expenses at cost with prior approval; 14-day payment terms.
- Catalogue prices for reference: C1 £14,000 · C2 £18,000 · C3 £24,000 · C4 £22,000 · C5 £20,000 · C6 £15,000 · C7 £9,500 · C8 £28,000 · C9 £26,000 · C10 £4,000 · A2 £4,000/month (all ESTIMATE).

Speaker note: two minutes; recommend one option and say why. Never discount on the call — guardrails in `/assets/PRICING_STRUCTURE.md` \$7 apply.

Visual: three cards (choice between siblings, brand rule 6); recommended card with an Ember eyebrow "Recommended"; prices in Manrope 700; "ESTIMATE, ex-VAT" in Slate on every card.

Slide 7 — Timeline

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- Week 0: kick-off (90 minutes), access and dependencies confirmed, interview list agreed.
- Weeks 1–[n]: Phase 1 — [milestone]. Check-in every [Tuesday].
- Weeks [n]–[n]: Phase 2 — [milestone]; interim readout.
- Week [n]: Phase 3 — final deliverables; acceptance review; handover of reusable assets.
- Week [n+2]: 30-minute follow-up call (included) to review adoption.
- Dependencies on you: [named counterpart availability; access to documents; interviewee introductions by week 1].

Speaker note: one minute. Point at the dependency line: most slips are caused by access, not by us.

Visual: Gantt-style table; Teal bars; Ember marker on the final acceptance date; dependency row in Slate.

Slide 8 — Who does the work and why trust us

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- [Founder name], Founder — [two-line background: KNOWN facts only, confirmed against the CV].
- Methods are documented and open: discovery guide, decision-scoring framework, DTAC readiness sequence, hazard-log starter (published in our Insights; see /docs/14_MARKETING_AND_CONTENT_ENGINE.md §6).
- Associates (if any): [name, role — with consent only; otherwise "an NHS-experienced associate will be proposed for your approval before Phase 1"].
- Advisers: [only if appointed with written consent; otherwise omit the line].
- Insurance: professional indemnity £[1m] and public liability in place [from date — do not present until purchased].
- What we don't have yet: named client references. We offer a first-engagement price review at the end of Phase 1 instead: if the interim readout does not meet the agreed acceptance criterion, you may stop at that point and pay only for Phase 1.

Speaker note: two minutes; the honesty about references is a strength when paired with the Phase-1 stop option — say it plainly.

Visual: one portrait panel (illustration placeholder until photography with consent); text block; no logos, no testimonials.

Slide 9 — Optional: where MaternaLink (in development) fits

Include only when the prospect's problem is maternity-related; otherwise delete.

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- Separately from this engagement, Vytalix is developing MaternaLink — a non-diagnostic care-coordination and communication platform for maternity services (NHS and low-connectivity settings). It does not diagnose, predict or score.
- It is not for sale today. It is in development; IP, prototype audit, clinical safety and DPIA gates are being closed first.
- If useful, we would welcome your team's input as a design partner in a service evaluation, with no purchase obligation and a separate, explicit agreement (/assets/PARTNERSHIP_DECK.md).
- Conflict rule: any consulting advice we give you on maternity digital tools declares this interest in writing.

Speaker note: one minute; this slide exists to disclose an interest, not to sell. If they want more, book a separate conversation.

Visual: Night background slide; persistent tag "Non-diagnostic — in development" in Ember Light; three lines only.

Slide 10 — Next step

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- Decision needed by: [date] (proposal validity 21 days from issue).
- Proposed kick-off: [date].
- What we need from you: [named counterpart; MSA and SOW signature; access list; 50% invoice paid or purchase order raised].
- What you get from us within two working days: the written proposal (`/assets/PROPOSAL_TEMPLATE.md`), the MSA, and a kick-off agenda.
- Contact: [founder email] · [LinkedIn] · [website]

Speaker note: two minutes; agree the decision date and the kick-off date on the call. Do not leave without one specific next action and a date.

Visual: Deep background; four lines; Ember CTA style on the decision line; footer "Vytalix — Technology for Life".

Using this deck

Rule	Detail
Never sent cold	Built from discovery notes; slide 2 must contain the prospect's own words
One recommendation	Slide 6 recommends one option; the others frame it
No claims that are not true	No logos, testimonials, "trusted by", named associates or advisers without consent; no MaternaLink outcome claims
Follow-up sequence	Same-day summary email; Day 2 proposal; Day 5 check-in call; Day 9 "what would help?"; Day 21 close (<code>/docs/13_SALES_ENGINE.md</code> §8)
Loss reasons coded	Price / timing / no budget / no need / chose competitor / no decision / not ICP — recorded in the CRM

Priorities / Risks / Next actions

Priorities

1. Lay the deck into the slide master and create a one-page "fill-in sheet" that maps discovery answers to the bracketed fields on slides 2, 3, 5, 6 and 7.
2. Use it on the first six prospects; record which option was chosen and which objections arose (feeds the pricing tests in `/docs/01_BUSINESS_MODEL.md` §8).
3. Purchase professional indemnity and public liability insurance before slide 8's insurance line is shown.

Risks

- Presenting a generic version without discovery; mitigation: slide 2 cannot be left with brackets — the deck is not sent if they remain.
- Discounting under pressure on the call; mitigation: guardrails in `/assets/PRICING_STRUCTURE.md` §7; any discount requires a written reason.
- Selling packages gated on capability not yet in place (C3, C5, C8, C9); mitigation: the "Proof required to sell" gate in `/assets/SERVICE_CATALOGUE.md`.

Next actions

- Founder, by Day 14: deck v1.0 in the slide master; fill-in sheet created; insurance quotes obtained.
- Founder, by Day 45: first two proposals issued from this deck; objections logged.
- Founder, by Day 90: option-selection and win/loss data reviewed; slides 5–6 revised.